NCCP 00361 · VERSION 4

Fulvestrant · injections every 4 weeks

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

> Clinician supportive-care discussion draft. This is not an issued prescription.

Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or G-CSF order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

Medicine-specific support

- Report injection pain with numbness or radiating leg pain, extensive bruising, jaundice or worsening symptoms.
- NCCP specifies 250 mg for Child-Pugh B, whereas EMA Faslodex product information describes no mild/moderate reduction with caution. The clinician and pharmacist must reconcile this source difference. Severe hepatic impairment is excluded.
- Assess personal fracture and osteoporosis risk. Fulvestrant does not carry an automatic AI DXA timetable; agree testing and repeat intervals when indicated.
- Assess persistent symptoms and prescribe individually; no automatic prophylactic antiemetic, steroid or G-CSF.

Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.2 · Sources checked 2026-10-18
Source reconciled; independent clinical, pharmacy and patient/PPI review pending